

Roche

The Shift From Reach to Real Engagement

A look at how Roche's marketing to specialist healthcare professionals has evolved, what is working, and where the next gain in engagement is genuinely sitting.

FOCUS AREAS

Oncology · Neurology · Cardiometabolic · Immunology

PREPARED

2026

WHAT THIS BRIEF COVERS

A concise read on how Roche's commercial marketing has moved from broad, rep led outreach toward a more data informed model, what the specialist audience is still missing, and where the clearest opportunity sits for closing that gap. Grounded in Roche's own public reporting and current pharmaceutical marketing research.

HCP Engagement

Content Strategy

AI & Personalization

Measurement

A Company Built on Long Horizons

Roche has spent more than a century building its reputation on evidence over hype, and that instinct still shapes how it talks to the specialists who prescribe its medicines and order its diagnostics. Understanding where that marketing model came from makes it much easier to see where it is heading next.

The traditional model

For most of its history, and still today in parts of the business, Roche's commercial engine ran on two pillars that worked well on their own but rarely talked to each other: broad awareness campaigns built around a drug launch, and in person rep detailing once a specialist was already close to a prescribing decision. It is a model built for a slower, less fragmented media environment, and specialists learned to expect Roche the same way they expected most pharmaceutical companies, correct on the science, generic in the delivery.

That generic delivery has become the single most measurable gap in pharmaceutical marketing today. Roughly eighty percent of healthcare professionals report that the communications they receive feel impersonal, while fewer than one in five say they feel genuinely, personally engaged by a pharmaceutical brand. Roche is not an outlier here. It is operating inside an industry wide pattern, which is exactly why closing that gap first would be such a meaningful differentiator.

~80% of specialists say outreach still feels generic and impersonal	<20% feel personally engaged despite rising marketing investment	CHF 30.4bn H1 2026 sales, up 6% at constant exchange rates
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Where Roche stands today

The present picture is genuinely more sophisticated, and parts of it are working well. A specialist facing content hub launched in 2025 has already produced a thirty percent lift in specialist lead generation along with strong organic traffic growth, built on clinical case studies rather than promotional copy. Roche's webinar program, delivered through ON24, has reported that seventy six percent of attendees changed their actual clinical practice after attending, which is one of the strongest content to outcome results anywhere in pharmaceutical marketing right now.

Behind the content, the infrastructure has changed too. In March 2026 Roche expanded its AI computing footprint to more than 3,500 high performance GPUs across the United States and Europe, one of the largest AI infrastructures owned by any pharmaceutical company, built in partnership with NVIDIA. Internally, platforms like Apollo now exist to unify data, and Roche has named a Global Head of Data, Analytics, AI and Marketing Technology, a strong signal that AI led personalization is a funded 2026 priority rather than an experiment sitting on the side.

What Actually Changed

Placed side by side, the shift in Roche's marketing model becomes much clearer. The channels have multiplied, but the more important change is in what the company is trying to prove to a specialist audience that has grown more skeptical, not less.

Dimension	The Earlier Model	The Current Model
Primary channel	In person rep detailing and print leave behinds, supported by broad digital awareness ads	Owned content hub, webinar series, and specialist facing digital platforms sitting alongside rep visits
Content focus	Product awareness and top line clinical claims aimed at a launch moment	Mechanism level explainers, real world case studies, and behavior change data from webinars
Personalization	Segmented mainly by specialty and geography, largely one size fits all within each group	AI driven segmentation ambition, backed by new infrastructure, though execution still lags the ambition
Technology backbone	Siloed systems across sales, marketing, and medical teams operating largely in parallel	A dedicated AI and MarTech leadership role, an internal data platform, and one of the largest GPU footprints in pharma
Proof of results	Impressions and reach, with limited visibility into what actually changed a prescribing decision	Named, public outcome metrics: thirty percent lead growth from the content hub, seventy six percent behavior change from webinars

The honest read on this shift

What stands out is not that Roche lacks ambition. It clearly does not. The gap between the two columns above is really a gap between infrastructure and operating discipline. The company has built strong individual formats and invested seriously in the technology that should enable true personalization, but coordination across sales, marketing, and medical teams is still described internally as unresolved, which is precisely what keeps a specialist experiencing five well funded initiatives as one inconsistent, uncoordinated relationship with the company.

Closing the Distance Between Ambition and Execution

Pharmaceutical marketing research is consistent on one point that applies directly here: spend is currently rising faster than the systems needed to spend it well. Seventy seven percent of pharma marketers plan to increase social investment, and sixty five percent are looking at AI driven personalization, while the underlying experience for most healthcare professionals has barely moved. Three areas stand out as the highest leverage next steps, each grounded in what is already proven to work rather than something unproven or speculative.

MID FUNNEL CONTENT

Build for the specialist who already knows the drug exists

Most pharmaceutical marketing, Roche included, is strong at the two ends of the journey: early awareness and late stage rep engagement. The middle, where a clinician has heard of a therapy but has not yet decided whether it belongs in their own practice, is where prescribing intent actually forms, and it is consistently under resourced across the industry.

Comparative effectiveness content, peer panel discussions, and real practice case studies belong here, and Roche's own webinar results already prove this format works when it gets built.

UNIFIED MEASUREMENT

Prove which touch actually moved a decision

Campaign data today lives scattered across ad platforms, CRM systems, speaker bureau databases, and prescription feeds, which makes it very difficult to say with confidence which piece of content or which channel actually influenced a prescribing choice. Industry research from Veeva has found that biopharma companies synchronizing their sales and marketing data see promotional effectiveness improve by roughly twenty three percent. For a company already investing in AI infrastructure at this scale, unifying that measurement layer is less a new cost and more a way of making the existing investment finally pay off.

FIRST PARTY TARGETING

Build around the data specialists actually give willingly

Specialists frequently move between hospital network devices and personal devices, and a meaningful share block cookies or use VPNs, which makes open web retargeting an increasingly unreliable signal. Webinar registrations, gated case study downloads, and logins to owned platforms are the opposite: a specialist choosing to hand over that information because the content was worth it. Building targeting around that kind of first party signal, rather than around a fading open web signal, is both more compliant and more accurate.

Why this moment matters

Roche has already spent on the hard part, the infrastructure, the AI capability, and the content formats that work. The remaining gap is smaller and more solvable than it looks: connecting what already exists so that a specialist experiences one coordinated, evidence led relationship instead of five well intentioned but disconnected ones. Few pharmaceutical companies are this close to solving the personalization problem the whole industry is chasing.